

1. Administrative & Study Identification

Full Study Title: Volrustomig Priming Regimens Exploratory Phase II Platform Study **Short Title/Acronym:** eVOLVE-01 **Official Regulatory Title:** A Phase II, Open-label Study to Evaluate the Safety, Tolerability, Pharmacokinetics, Immunogenicity, and Antitumor Activity of Volrustomig Priming Regimens in Combination With Other Anticancer Agents in Participants With Solid Tumors (eVOLVE-01) **Protocol Number:** eVOLVE-01 **NCT Number:** NCT06448754 **Posting ID:** 515765215473271155 **Sponsor/Company Name:** AstraZeneca **Investigational Product:** Volrustomig (MEDI5752) administered in combination with chemotherapy (carboplatin, pemetrexed, paclitaxel, and ramucirumab) **Phase of Development:** Phase II **Trial Status:** RECRUITING (Currently recruiting participants) **Medical Monitor:** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To assess the safety, tolerability, and antitumor activity—specifically the Confirmed Objective Response Rate (ORR)—of volrustomig in combination with other anticancer drugs in participants with specified solid tumors. **Secondary Objectives:** To evaluate the Disease Control Rate (DCR), Duration of Response (DoR), Progression-Free Survival (PFS), Overall Survival (OS), pharmacokinetics (PK), and immunogenicity (anti-drug antibodies).

2.2 Background & Rationale To address the need for improved first-line therapeutic outcomes in patients with metastatic non-small cell lung cancer (mNSCLC). The study evaluates whether adding the investigational immunotherapy volrustomig (in varying dosing and priming regimens) to standard histology-specific chemotherapy provides a tolerable safety profile and superior clinical efficacy.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional (Platform Study) **Control Type:** Active Comparator (Different volrustomig regimens + standard chemotherapy) **Blinding:** Open-label **Randomization:** Randomized (Substudy 1 uses 1:1 randomization between Arm 1A and 1B; Substudy 2 enrolls to a single Arm 2A)

3.2 Planned Enrollment & Duration Target \$N\$: 180 participants **Number of Sites:** Approximately 67 research sites globally

Estimated Study Start: 27-Aug-2024 **Estimated Primary Completion:** 30-Jul-2027

4. Subject Selection (Eligibility Criteria)

4.1 Key Inclusion Criteria 1. Age ≥ 18 years. 2. Eastern Cooperative Oncology Group (ECOG) performance status of 0 or 1 with no deterioration. 3. Life expectancy ≥ 12 weeks, adequate organ and bone marrow function, and body weight > 35 kg at screening and randomization. 4. Histologically or cytologically documented non-squamous (NSQ) NSCLC in Substudy 1, and squamous (SQ) or NSQ mNSCLC in Substudy 2. 5. Absence of sensitizing epidermal growth factor receptor (EGFR) mutations and absence of documented tumor genomic alteration results from tests conducted as part of standard local practice in any other actionable driver oncogenes. 6. At least one measurable lesion not previously irradiated that can be accurately measured at baseline as ≥ 10 mm in the longest diameter.

4.2 Key Exclusion Criteria 1. Prior chemotherapy or any other systemic therapy for Stage IV NSCLC. Participants who have received prior platinum-containing adjuvant, neoadjuvant, or definitive chemoradiation for local disease are eligible, provided that progression has occurred > 12 months from the end of the last therapy. 2. Spinal cord compression or history of primary active immunodeficiency. 3. Active or prior documented autoimmune or inflammatory disorders. 4. Mixed small-cell lung cancer and NSCLC histology or sarcomatoid variant. 5. Brain metastases unless asymptomatic, stable, and not requiring steroids for at least 14 days prior to the start of study intervention. A minimum of 2 weeks must have elapsed between the end of radiation therapy and study enrollment.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * **Substudy 1 (Arms 1A & 1B):** Volrustomig priming dose followed by volrustomig dosing regimen 1 or 2, in combination with carboplatin and pemetrexed. * **Substudy 2 (Arm 2A):** Volrustomig priming dose followed by volrustomig dosing regimen 2, in combination with ramucirumab and histology-specific chemotherapy (carboplatin + pemetrexed or paclitaxel).

(Note: Carboplatin [Trade Name: Paraplatin; ATC Code: L01XA02] is included as part of the chemotherapy regimen.)

Route of Administration: Intravenous (IV) infusion for volrustomig and all chemotherapies. **Duration of Treatment:** Up to 2 years and 10 months.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for managing chemotherapy-related toxicities (e.g., antiemetics). **Prohibited:** Concurrent investigational agents, live attenuated vaccines, and any prior systemic therapy for Stage IV NSCLC (except as noted in exclusion criteria).

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to -1	Informed Consent, Medical History, Tumor Imaging, Eligibility Labs
Baseline	Day 0	Randomization, First IV Dose, Baseline PK/ADA
Interim	Approx. 16 visits	Onsite Visits: IV Infusions, Safety Labs, Efficacy Assessment (RECIST 1.1), PK Tracking
End of Study	Up to 2 yr 10 mo	Final Tumor Assessment, Exit Interview, Survival Follow-up Phone Call

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: 1. Number of participants with Adverse Events (AEs) and Serious Adverse Events (SAEs) to assess safety and tolerability. 2. Confirmed Objective Response Rate (ORR). **Measurement Method:** ORR is defined as the percentage of participants who have a confirmed Complete Response (CR) or confirmed Partial Response (PR) assessed via Response Evaluation Criteria in Solid Tumors, Version 1.1 (RECIST 1.1).

7.2 Secondary & Exploratory Endpoints 1. Disease Control Rate (DCR). 2. Duration of Response (DoR). 3. Progression-Free Survival (PFS) and Overall Survival (OS). 4. Pharmacokinetics (AUC, trough concentrations) and immunogenicity (incidence of Anti-Drug Antibodies).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring during the 16 onsite trial visits and remote follow-ups. **Serious Adverse Events (SAEs):** Captured continuously from screening (Day -28) up to 2 years and 10 months. **Data Monitoring Committee (DMC):** Independent safety monitoring to review dose-limiting toxicities and tolerability of the platform study combinations.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for primary efficacy (ORR); Safety Set for all AE/SAE evaluations. **Sample Size Justification:** Target $N=180$ powered to assess the preliminary efficacy (ORR) and establish the safety profile of the varying

volrustomig dosing regimens across Substudies 1 and 2. **Handling of Missing Data:** Standard oncology guidelines (e.g., right-censoring for time-to-event endpoints like PFS and OS).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Routine onsite and remote monitoring across the 67 global research sites. **Audit Trail:** Standardized electronic Case Report Forms (eCRFs) locking and central radiological reviews for RECIST 1.1 confirmation.

11. Study Identifiers & Governance

Primary ID: eVOLVE-01 **Registry Number:** NCT06448754 **Posting ID:** 515765215473271155 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** Volrustomig Priming Regimens Exploratory Phase II Platform Study **Official Regulatory Title:** A Phase II, Open-label Study to Evaluate the Safety, Tolerability, Pharmacokinetics, Immunogenicity, and Antitumor Activity of Volrustomig Priming Regimens in Combination With Other Anticancer Agents in Participants With Solid Tumors (eVOLVE-01)

12. Clinical Framework (Oncology Specific)

Disease Areas: Non-small Cell Lung Cancer **Primary Condition:** Metastatic Non-Small Cell Lung Cancer (mNSCLC) **Preferred UMLS Name:** Non-Small Cell Lung Carcinoma **Semantic Type:** Neoplastic Process **MeSH Code:** D016938 **SNOMED ID:** 457721000124104 **Disease Definition:** Stage IV includes: (Any T, Any N, M1a); (Any T, Any N, M1b). M1a: Lung cancer with separate tumor nodule(s) in a contralateral lobe; tumor with pleural nodules or malignant pleural (or pericardial) effusion. M1b: Distant metastasis. (AJCC 7th ed.) **Diagnosis Threshold:** Histologically or cytologically documented Stage IV NSQ or SQ NSCLC without actionable driver mutations (e.g., EGFR) **Medical Methodology:** Investigational immunotherapy (Volrustomig) + Standard Chemotherapy (Carboplatin, Pemetrexed, Paclitaxel) **Optimization Target:** Maximum Confirmed Objective Response Rate (ORR) and acceptable safety profile

13. Study Procedures & Methodology

Management Pathway: Advanced NSCLC 1L chemo-immunotherapy pathway **Education Protocol (HCP):** Site initiation visits, RECIST 1.1 evaluation criteria training, and IV infusion protocol guidelines **Education Protocol (Patient):** Informed consent, treatment

scheduling, and AE self-reporting (e.g., recognizing immune-mediated toxicities) **Quality Audit Mechanism:** Central imaging review & standardized safety data tracking

14. Observation & Follow-Up Schedule

Total Duration: Up to 2 years and 10 months **Onsite Visit Cadence:** Approximately 16 trial site visits during the treatment phase
Remote Monitoring Frequency: Follow-up phone calls post-treatment
Checklist Review Interval: During each treatment cycle prior to IV infusion

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead	
Statistician	[Name]	_____	[DD-MMM-YYYY]				